

Hypertension

History

- Does patient have a history of hypertension?
- Has the patient taken his/her medications today?
- What are patient's other medical problems?
- Is patient having symptoms of hypertension or is this an incidental finding? When did symptoms start?
- What medications does patient take? (Nitroglycerin, blood thinners, NSAIDs, beta blockers, chlorpromazine, Reglan, Topamax, Imitrex, triptans, etc.?)
- What are patient's symptoms? (Is patient having a hypertensive emergency?)

Physical

- General appearance – In distress? Agitated? Confused? Comatose?
- Vital Signs – Include blood pressure evaluation in BOTH arms (if different consider thoracic aortic aneurysm)
- EKG – Abnormalities? STEMI?
- Breath sounds – Crackles / rales / wheezing (signs of pulmonary edema?)
- Abdomen – Bruits? Palpable / pulsating mass (AAA)?
- Neuro – Mental status? Focal findings (FAST stroke assessment and VAN stroke assessment)?
- Determine if patient meets criteria for Uncontrolled Hypertension, Hypertensive Urgency, or Hypertensive Emergency

Uncontrolled Hypertension

- Patient has elevated BP with no signs or symptoms of end organ damage
- Patient needs BP control, but not urgently or emergently

Hypertensive Urgency

The patient has an elevated BP with some mild signs / symptoms:

- Dizziness
- Lightheadedness
- Mild headache

These patients are not in distress, and do not have signs of End-Organ Damage as seen in Hypertensive Emergency. They need urgent, but not emergent, treatment. EMS should monitor for worsening symptoms.

Hypertensive Emergency

Signs of End Organ Damage:

- Neurologic – Altered mental status, stroke-like symptoms, intracranial hemorrhage, severe headache
- Cardiovascular – myocardial infarction, chest pain / angina, signs of CHF / pulmonary edema, AAA
- Other – kidney damage, eclampsia, retinopathy (vision loss)

EMT/AEMT

- Follow General Medical Care Guideline
- Follow Adult Resuscitation Guideline, as indicated
- Assess for signs of stroke
- If the patient has not yet taken his/her medications for the day, allow the patient to take his/her medications

Paramedic

SBP > 220 OR DBP > 120 WITH signs of end-organ damage (chest pain, shortness of breath (without signs of heart failure), altered mental status

- Consider **Metoprolol** 5 mg IO/IV every 5 minutes x 3, max dose 15 mg

Neurologic, cardiovascular or obstetrical concern

- Treat per appropriate Guideline